



NCI Protocol #: 10323

Patient ID:

Date:

1 of 5

Patient Name:
Date Collected:
Biopsy Site: **Bone Marrow**
Sample ID:
Telephone:
Tumor Content (%): **50%**

Patient DOB:
Referring Physician:
Primary Diagnosis: **Multiple Myeloma**
MoCha ID:
Fax:

Preanalytics to include tumor enrichment as required is performed by Roswell Park Comprehensive Cancer Center; Buffalo; NY 14203

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Relevant Biomarkers

Tier	Genomic Alteration	Relevant Therapies (In this cancer type)	Relevant Therapies (In other cancer type)	Clinical Trials
IID	KRAS Q22K	None	None	4

Public data sources included in relevant therapies: FDA1, NCCN

Tier Reference: Li et al. *Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer: A Joint Consensus Recommendation of the Association for Molecular Pathology, American Society of Clinical Oncology, and College of American Pathologists.* J Mol Diagn. 2017 Jan;19(1):4-23.

Prevalent cancer biomarkers without relevant evidence based on included data sources

MAX R36W

Variant Details

DNA Sequence Variants

Gene	Amino Acid Change	Coding	Variant ID	Locus	Allele Frequency	Coverage	Transcript	Variant Effect
KRAS	p.(Q22K)	c.64C>A	.	chr12:25398255	11.82%	1997	NM_033360.3	missense
MAX	p.(R36W)	c.106A>T	.	chr14:65560491	19.40%	2000	NM_002382.4	missense

Comments:

N/A

Shahanawaz Jiwani, MD, PhD, Laboratory Director | CLIA ID 21D2097127

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.10(005). The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

Report Signed By

Name	Role	Date

Biomarker Descriptions

KRAS (KRAS proto-oncogene, GTPase)

Background: The KRAS proto-oncogene encodes a GTPase that functions in signal transduction and is a member of the RAS superfamily which also includes NRAS and HRAS. RAS proteins mediate the transmission of growth signals from the cell surface to the nucleus via the PI3K/AKT/MTOR and RAS/RAF/MEK/ERK pathways, which regulate cell division, differentiation, and survival^{1,2,3}.

Alterations and prevalence: Recurrent mutations in RAS oncogenes cause constitutive activation and are found in 20-30% of cancers. KRAS mutations are observed in up to 10-20% of uterine cancer, 30-35% of lung adenocarcinoma and colorectal cancer, and about 60% of pancreatic cancer⁴. The majority of KRAS mutations consist of point mutations occurring at G12, G13, and Q61^{4,5,6}. Mutations at A59, K117, and A146 have also been observed but are less frequent^{7,8}.

Potential relevance: The KRAS inhibitor, sotorasib⁹, is approved (2021) for the treatment of adult patients with KRAS G12C-mutated locally advanced or metastatic non-small cell lung cancer (NSCLC). The FDA has granted breakthrough therapy designation (2021) to the small molecule inhibitor, adagrasib, for KRAS G12C positive in non-small cell lung cancer following prior systemic therapy¹⁰. The small molecule inhibitor, RO-5126766, was also granted breakthrough designation (2021) alone for KRAS G12V mutant non-small cell lung cancer or in combination with defactinib, for KRAS mutant endometrial carcinoma and KRAS G12V mutant non-small cell lung cancer¹¹. Additionally, onvansertib¹² was granted fast track designation (2020) for second-line treatment of patients with KRAS-mutated metastatic colorectal cancer (mCRC). The EGFR antagonists, cetuximab¹³ and panitumumab¹⁴, are contraindicated for treatment of colorectal cancer patients with KRAS mutations in exon 2 (codons 12 and 13), exon 3 (codons 59 and 61), and exon 4 (codons 117 and 146)⁸. Additionally, KRAS mutations are associated with poor prognosis in NSCLC¹⁵.

MAX (MYC associated factor X)

Background: The MAX gene encodes the MYC associated factor X protein, a member of the basic helix-loop-helix leucine zipper (bHLHZ) transcription factor family, which also includes MNT and MXD1-4¹⁶. MAX is ubiquitously expressed as two common isoforms, p21 and p22, each of which have unique DNA binding and biological activities¹⁶. MAX heterodimerizes with bHLHZ transcription factors including MYC, MAD, MNT, and MGA to form complexes that act on DNA sequences to regulate the transcription of target genes involved in cell growth and proliferation^{16,17}. Homozygous alterations involving MAX lead to a protein incapable of dimerization and repressing transcription¹⁸. Germline mutations in MAX are observed in hereditary pheochromocytoma, a rare neural crest cell tumor¹⁸.

Alterations and prevalence: Somatic mutations in MAX are observed in 4% of uterine cancer, and 1-1.5% of lung adenocarcinoma, colorectal cancer, and clear cell renal cell carcinoma^{4,7}. The missense mutation R60Q has been observed to be recurrent in a variety of cancers^{4,7,19}. This mutation falls within the bHLH domain and has been observed to alter the DNA binding properties of MAX¹⁹. Amplifications are observed 2% of diffuse large B-cell lymphoma^{4,7}.

Potential relevance: Currently, no therapies are approved for MAX aberrations.

Relevant Therapy Summary

☒ In this cancer type

☐ In other cancer type

☒ In this cancer type and other cancer types

☒ No evidence

KRAS Q22K			
Relevant Therapy	FDA	NCCN	Clinical Trials*
ixazomib, pomalidomide, steroid, cobimetinib	×	×	● (I/II)
AZD-0364	×	×	● (I)
GGTI-2418, bortezomib	×	×	● (I)
trametinib	×	×	● (I)

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

Clinical Trials Summary

KRAS Q22K		
NCT ID	Title	Phase
NCT03732703	Myeloma-Developing Regimens Using Genomics (MyDRUG) (Genomics Guided Multi-arm Trial of Targeted Agents Alone or in Combination With a Backbone Regimen)	I/II
NCT03091257	An Open-label, Pilot Study of Dabrafenib and/or Trametinib in Patients With Relapsed and/or Refractory Multiple Myeloma	I
NCT03900442	Phase Ib Pharmacodynamic and Pharmacokinetic Study of the Geranylgeranyltransferase I Inhibitor PTX-100 (GGTI-2418) in Patients With Advanced Malignancies	I
NCT04305249	A Phase I, Open-Label, Multi-Center Dose Finding Study to Investigate the Safety, Pharmacokinetics, and Preliminary Efficacy of ATG-017 Monotherapy in Patients With Advanced Solid Tumors and Hematological Malignancies	I

Assay Information

Methodology, Scope, and Application:The OCAv3 next-generation sequencing (NGS) assay identifies more than 3000 annotated mutations of interest (MOIs) broadly categorized into 5 mutation types: single nucleotide variants (SNVs), small insertions/deletions (Indels), large (>3 bases) insertions/deletions (Large Indels), copy number variants (CNVs), and gene fusions. The assay utilizes the Thermo Fisher Scientific Oncomine® Comprehensive Assay v3.0 (OCAv3), a next-generation sequencing (NGS) assay that utilizes a multiplex polymerase chain reaction (PCR) with DNA and RNA extracted from formalin-fixed tissue for sequencing on the Ion S5 XL platform and analyzed by the current version of Torrent Suite Software and Ion Reporter. The OCAv3 NGS Assay currently can reliably identify the presence or absence of >3000 known mutations and polymorphisms in the 161 unique genes compared to the Human Reference Genome hg19, including the genes listed below. The OCAv3 assay is a laboratory developed test designed to find gene mutations within tumors (somatic mutations).

Analytical Sensitivity and Specificity:The OCAv3 NGS Assay has been determined to be suitably analytically sensitive and specific for the various types of abnormalities within its reportable range, demonstrating 97.18% sensitivity compared with orthogonal assay results and 100% specificity for the 5 mutation types reported. 99.99% or greater reproducibility has been demonstrated. All quality measures for this assay were within defined assay parameters.

Shahanawaz Jiwani, MD, PhD, Laboratory Director | CLIA ID 21D2097127

Hereditary and Germline Mutations: This assay examines tumor tissue only and does not examine normal (non-tumor) tissue. Mutations detected by the assay may be present only in the tumor, or in every cell of the body (including non-tumor cells). This test also cannot tell whether a potential germline mutation causes or will cause a hereditary cancer syndrome. If the patient's history includes any one or combination of features suggestive of a possible hereditary cancer predisposition (such as, cancer arising at a young age, especially a cancer of a type unusual in younger patients; a personal history of multiple different types of cancers; or a significant cancer history among blood relatives, especially but not exclusively of the same type as the patient's) it is recommended that the physician arrange for the patient to meet with a genetic counselor and, if warranted, undergo the appropriate genetic test on normal (i.e. non-tumor) tissue (blood or cells brushed from the oral surface of the cheek) to check for a germline abnormality, regardless of the results of this research study.

Report Content: Thermo Fisher Scientific's Ion Torrent OncoPrint Reporter software was used in the generation of this report. Software was developed and designed internally by Thermo Fisher Scientific. The analysis was based on the current version of OncoPrint Reporter. The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. FDA information was sourced from www.fda.gov and is current as of this version. NCCN information was sourced from www.nccn.org and reflects its current version. Clinical Trials information reflects the current version. For the most up-to-date information regarding a particular trial, search www.clinicaltrials.gov by NCT ID or search local clinical trials authority website by local identifier listed in 'Other Identifiers.' Variants are reported according to HGVS nomenclature and classified following AMP/ASCO/CAP guidelines (Li et al. 2017). Based on the data sources selected, variants, therapies, and trials identified in this report are listed in order of potential clinical significance but not for predicted efficacy of the therapies.

Genes assayed for hotspot mutations:

AKT1, AKT2, AKT3, ALK, AR, ARAF, AXL, BRAF, BTK, CBL, CCND1, CDK4, CDK6, CHEK2, CSF1R, CTNNB1, DDR2, EGFR, ERBB2, ERBB3, ERBB4, ERCC2, ESR1, EZH2, FGFR1, FGFR2, FGFR3, FGFR4, FLT3, FOXL2, GATA2, GNA11, GNAQ, GNAS, H3F3A, HIST1H3B, HNF1A, HRAS, IDH1, IDH2, JAK1, JAK2, JAK3, KDR, KIT, KNSTRN, KRAS, MAGOH, MAP2K1, MAP2K2, MAP2K4, MAPK1, MAX, MDM4, MED12, MET, MTOR, MYC, MYCN, MYD88, NFE2L2, NRAS, NTRK1, NTRK2, NTRK3, PDGFRA, PDGFRB, PIK3CA, PIK3CB, PPP2R1A, PTPN11, RAC1, RAF1, RET, RHEB, RHOA, ROS1, SF3B1, SMAD4, SMO, SPOP, SRC, STAT3, TERT, TOP1, U2AF1, XPO1

Genes assayed with full exon coverage:

ARID1A, ATM, ATR, ATRX, BAP1, BRCA1, BRCA2, CDK12, CDKN1B, CDKN2A, CDKN2B, CHEK1, CREBBP, FANCA, FANCD2, FANCI, FBXW7, MLH1, MRE11, MSH2, MSH6, NBN, NF1, NF2, NOTCH1, NOTCH2, NOTCH3, PALB2, PIK3R1, PMS2, POLE, PTCH1, PTEN, RAD50, RAD51, RAD51B, RAD51C, RAD51D, RB1, RNF43, SETD2, SLX4, SMARCA4, SMARCB1, STK11, TP53, TSC1, TSC2

Genes assayed for copy number variations:

AKT1, AKT2, AKT3, ALK, AR, AXL, BRAF, CCND1, CCND2, CCND3, CCNE1, CDK2, CDK4, CDK6, EGFR, ERBB2, ESR1, FGF19, FGF3, FGFR1, FGFR2, FGFR3, FGFR4, FLT3, IGF1R, KIT, KRAS, MDM2, MDM4, MET, MYC, MYCL, MYCN, NTRK1, NTRK2, NTRK3, PDGFRA, PDGFRB, PIK3CA, PIK3CB, PPARG, RICTOR, TERT

Genes assayed for detection of fusions:

AKT2, ALK, AR, AXL, BRAF, BRCA1, BRCA2, CDKN2A, EGFR, ERBB2, ERBB4, ERG, ESR1, ETV1, ETV4, ETV5, FGFR1, FGFR2, FGFR3, FGF, FLT3, JAK2, KRAS, MDM4, MET, MYB, MYBL1, NF1, NOTCH1, NOTCH4, NRG1, NTRK1, NTRK2, NTRK3, NUTM1, PDGFRA, PDGFRB, PIK3CA, PPARG, PRKACA, PRKACB, PTEN, RAD51B, RAF1, RB1, RELA, RET, ROS1, RSP02, RSP03, TERT

DISCLAIMER

This assay is considered a Laboratory Developed Test (LDT). Its performance characteristics have been determined through extensive testing although it has not been cleared by the US Food and Drug Administration and such approval is not required for clinical implementation. Furthermore, any comments included in the report are strictly interpretive and the opinion of the reviewer. This laboratory is certified under the Clinical Laboratory Improvements Amendments (CLIA) for the performance of high-complexity testing for clinical purposes. The correlative data presented is a result of the curation of published data sources, but may not be exhaustive. The content of this report has not been evaluated or approved by the FDA or other regulatory agencies.

References

1. Pylayeva-Gupta et al. RAS oncogenes: weaving a tumorigenic web. *Nat. Rev. Cancer*. 2011 Oct 13;11(11):761-74. PMID: 21993244
2. Karnoub et al. Ras oncogenes: split personalities. *Nat. Rev. Mol. Cell Biol.* 2008 Jul;9(7):517-31. PMID: 18568040
3. Scott et al. Therapeutic Approaches to RAS Mutation. *Cancer J.* 2016 May-Jun;22(3):165-74. doi: 10.1097/PPO.000000000000187. PMID: 27341593
4. Weinstein et al. The Cancer Genome Atlas Pan-Cancer analysis project. *Nat. Genet.* 2013 Oct;45(10):1113-20. PMID: 24071849
5. Román et al. KRAS oncogene in non-small cell lung cancer: clinical perspectives on the treatment of an old target. *Mol Cancer*. 2018 Feb 19;17(1):33. doi: 10.1186/s12943-018-0789-x. PMID: 29455666
6. Dinu et al. Prognostic significance of KRAS gene mutations in colorectal cancer--preliminary study. *J Med Life*. 2014 Oct-Dec;7(4):581-7. PMID: 25713627
7. Cerami et al. The cBio cancer genomics portal: an open platform for exploring multidimensional cancer genomics data. *Cancer Discov*. 2012 May;2(5):401-4. PMID: 22588877
8. Allegra et al. Extended RAS Gene Mutation Testing in Metastatic Colorectal Carcinoma to Predict Response to Anti-Epidermal Growth Factor Receptor Monoclonal Antibody Therapy: American Society of Clinical Oncology Provisional Clinical Opinion Update 2015. *J. Clin. Oncol.* 2016 Jan 10;34(2):179-85. PMID: 26438111
9. https://www.accessdata.fda.gov/drugsatfda_docs/label/2021/214665s000lbl.pdf
10. <https://ir.mirati.com/press-releases/press-release-details/2021/Mirati-Therapeutics-Adagrasib-Receives-Breakthrough-Therapy-Designation-from-U.S.-Food-and-Drug-Administration-for-Patients-with-Advanced-Non-Small-Cell-Lung-Cancer-Harboring-the-KRAS-G12C-Mutation/default.aspx>
11. <https://investor.verastem.com/news-releases/news-release-details/verastem-oncology-receives-breakthrough-therapy-designation-vs>
12. <https://cardiffoncology.investorroom.com/2020-05-28-Cardiff-Oncology-Announces-Fast-Track-Designation-Granted-by-the-FDA-to-Onvansertib-for-Second-Line-Treatment-of-KRAS-Mutated-Colorectal-Cancer>
13. https://www.accessdata.fda.gov/drugsatfda_docs/label/2021/125084s279lbl.pdf
14. https://www.accessdata.fda.gov/drugsatfda_docs/label/2021/125147s210lbl.pdf
15. Slebos et al. K-ras oncogene activation as a prognostic marker in adenocarcinoma of the lung. *N. Engl. J. Med.* 1990 Aug 30;323(9):561-5. PMID: 2199829
16. Hurlin et al. The MAX-interacting transcription factor network. *Semin. Cancer Biol.* 2006 Aug;16(4):265-74. PMID: 16908182
17. Susan. An Overview of the Basic Helix-Loop-Helix Proteins. *Genome Biol.* 2004;5(6):226. PMID: 15186484
18. Cascón et al. MAX and MYC: a heritable breakup. *Cancer Res.* 2012 Jul 1;72(13):3119-24. PMID: 22706201
19. Wang et al. MAX is an epigenetic sensor of 5-carboxylcytosine and is altered in multiple myeloma. *Nucleic Acids Res.* 2017 Mar 17;45(5):2396-2407. PMID: 27903915